



PROTOCOL

Specialised CAMHS MST Programme Entry Protocol

Scope (Staff):	All Staff
Scope (Area):	Multisystemic Therapy (MST) Programme

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This protocol is currently under review to be incorporated into the relevant Model of Care.
The content remains current throughout this review process.

Aim

To describe the entry process of the Multisystemic Therapy (MST) Programme within the Specialised CAMHS directorate, including eligibility, referral selection, and process for facilitating access to services that may better meet needs of the family and child.

Risk

A child may not gain entry to a service that meets their specific needs in a timely manner.

Background

MST was implemented in WA Health in 2005 to service an identified population of children and youth with history of severe behavioural problems, typically unsuited for clinic-based intervention. Most referrals to MST are from government departments, non-government agencies, Perth Children's Hospital, CAMHS clinics, and private professionals. The MST management team and nominated senior mental health professionals from referring departments are involved in the selection of the most appropriate referrals for activation by the two small MST clinical teams.

Multisystemic Therapy is a leading international, evidence-based intervention, specifically designed to help families with children aged 11-16 years, experiencing severe behavioural problems. Specialist clinicians deliver an intensive, tailored, family intervention over a three to five-month period. The intervention involves clinicians visiting the young person's home three times per week to help parents communicate more effectively, problem-solve, positively influence and motivate improved behaviour. The intervention is highly-flexible and tailored to family needs. Parents are supported

by phone 24/7 during this intensive intervention to help manage periods of behaviour escalation and crisis. The MST intervention is designed to help families with children primarily experiencing externalising behaviours (disruptive behavioural disorders). Children without a stable home environment and/or stable primary carer, or children with a developmental disorder (e.g. autism), or history of self-harm or suicidal intent within the previous 6 months, are not suitable for MST.

Definitions

Access: ability of consumers or potential consumers to obtain required or available services when needed and within an appropriate time.

Entry: the process provided by the mental health service which assists the consumer and their carers to make contact with the mental health service and receive appropriate assistance.

Child: an infant, child, adolescent or young person under the age of 18 years.

Parent: any parent, legal guardian or identified carer of the child. For a child of Aboriginal or Torres Strait Islander descent, this includes a person within their kinship group who is recognised as their carer under customary laws and traditions.

Therapist Adherence Measure (TAM): a standardised measure of a therapist's adherence to the MST model as reported by the primary caregiver of the family.

Principles / Key points

- The CAMHS MST programme is currently comprised of two small clinical teams located in Fremantle and Hillarys, that service families located in the Perth metropolitan area.
- The programme is an intensive outreach service with flexible hours of operation to enable regular clinical meetings with parents and carers in their homes.
- Limited MST service capacity, coupled with the intensive nature of the intervention, requires careful consideration of referral submission by referrers, including prior discussion with MST staff and/or referral managers, then referral selection process by the MST Referral Review Committee (RRC).
- Referrals are received from a wide range of sources and prioritised for acceptance and activation by the RRC based on suitability for the MST intervention model, team capacity and symptom severity.
- For client safety and intervention effectiveness, it is essential that referral forms are completed accurately and comprehensively.
- Information regarding referral suitability and submission process can be obtained from the Fremantle MST office or dedicated MST referral managers based within various government departments. Names and contact numbers for the MST referral managers are available from the Fremantle MST office.

Service Information

- Those persons **internal** to WA Health can access information about MST (including the referral form) via the Child and Adolescent Health Service [MST Intranet page](#).
- Those persons **external** to WA Health can access information about MST via the [MST Internet page](#).
- Both **internal and external** parties can obtain further information about MST by emailing CAMHS.MST@health.wa.gov.au. Staff can provide copies of the MST Referral Form, Brochure, Inclusion and Diversion Criteria, and Information about MST for Families
- If a prospective referrer has access to a MST Referral Manager within their department, they should contact that person to first discuss referral suitability and service capacity, and to obtain the MST Referral form. Current MST Referral Managers are included on the front page of the MST referral form.

Process

1. Review of referrals

All referrals received are reviewed and prioritised by the MST Referral Review Committee (RRC). The RRC is comprised of MST program and team managers, and senior mental health professionals from government departments regularly referring to MST, with the task of determining referrals most likely to benefit.

- **Accepted referrals**

Referrers of families who are considered either unsuitable (ineligible) for the MST intervention, or insufficiently severe to be likely offered a place in the program in the near future, are promptly contacted by the relevant MST team manager and provided with information about alternate services. These referrals will still be considered at the next two RRC meetings, however typically more severe referrals will be prioritised.

Following referral selection and prioritisation after the RRC meeting, the referrer is notified and client's family contacted to arrange an initial visit to the family. The initial visit provides an opportunity to fully explain the MST intervention and process to the family, then obtain their informed consent. The parent chooses which of their child's problematic behaviours they wish to focus on improving, and negotiate when they want the clinician to visit. Visits often occur outside normal business hours to ensure that both parents/carers (where applicable) are involved and aligned with the intervention procedure. Clinicians also liaise with the child's school, and significant others in the community, to ensure that all relevant "systems" influencing the child are involved.

2. Privacy and confidentiality

The relevant MST team manager and allocated clinician meet with families to explain the MST intervention before seeking consent to treatment. Staff explain to the family that their private information is securely maintained by the service and

only shared when necessary (i.e. safety concerns) with those services noted on the consent form.

3. Engagement with children, families and stakeholders

Throughout the MST intervention, in addition to intervention contact, families are contacted bi-monthly by an MST staff member not directly involved in their clinical care, and asked to provide feedback about the clinician's work with the family, by completing a TAM (Therapist Adherence Measure) questionnaire. This valuable client feedback measures each clinician's fidelity to the MST intervention model, and thereby able to identify possible clinician training and development needs.

4. Monitoring and management of demand and capacity to ensure access in a timely manner.

All professionals who submit referrals are informed by the relevant MST team manager of the suitability of the referral following the initial RRC meeting. Suitable referrals not selected for activation at the first RRC meeting are maintained for re-consideration at the next two RRC meetings. If referrals are not selected after three attempts, relevant team manager will suggest referrers consider alternate services. MST has limited service capacity for high prevalence child & adolescent disorders, so doesn't utilise a waitlist model to manage referrals not selected within 3 months. Service demand is mostly controlled by dedicated referral managers who determine the number of referrals they submit to the RRC from their respective Departments.

5. Integration and partnership

The MST Programme maintains strong working relationships with all stakeholders, and encourages participation and representation of stakeholders on the MST RRC and the MST Referral Manager Committee. This professional collaboration ensures appropriate communication and screening of referrals, and significantly reduces the number of inappropriate referrals, and a potentially large number of lower-severity referral submissions. This professional collaboration also ensures that confidential information about families (e.g. complicating factors, relevant safety information, etc.) is shared between services when appropriate.

Related CAHS internal policies, procedures and guidelines

[CAMHS Patient Rights Under The Mental Health Act](#)

References and related external legislation, policies, and guidelines (if required)

The [Vancouver style](#) referencing is as per CAHS Library and Information Service.

This document can be made available in alternative formats on request.

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